

Document info

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MRI Brain WO/W Contrast

Patient:	KAREN CLAYBORNE	DOB:	Jan 11, 1966
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REPORT

MRI Brain WO/W Contrast

CLINICAL HISTORY: 59 years Female stage IV lung cancer Evaluate cerebellar lesion/other mets?

COMPARISONS: CT head April 1, 2025

TECHNIQUE: Multiplanar, multisequence images were obtained through the brain without and with contrast.

FINDINGS: There is a solid and cystic mass of the cerebellar vermis with heterogenous enhancing solid components measuring 3.1 x 2.4 x 3 cm. This is favored to be intraparenchymal rather than intraventricular, but does involve the ependymal surface of the fourth ventricle and results in severe mass effect completely effacing the fourth ventricle. There is mild dilation of the lateral and third ventricles, which appears similar to the prior exam. There is severe edema of the upper cerebellar vermis and moderate edema of the medial portions of the cerebellar hemispheres. There is crowding of the posterior fossa with partial prepontine cistern effacement. Cerebellar tonsils appear slightly low-lying extending 3 mm inferior to the basion-chest opisthion line. Edema of the upper cerebellar vermis partially effaces the supracerebellar cistern. There is no evidence of midline shift. No abnormal extra axial fluid collections are seen. There is

minimal periventricular white matter T2/FLAIR hyperintensity may indicate chronic macrovascular white matter disease. No other abnormal enhancing lesions are identified in the cranium. There is no evidence of diffusion restriction. Orbital contents are unremarkable. There are no air-fluid levels the paranasal sinuses. There is moderate rightward deviation the nasal septum. Mastoid air cells are clear. No calvarial marrow lesions are seen.

IMPRESSION:

1. Solid and cystic mass of the cerebellar vermis with heterogenous enhancing solid components measuring 3.1 x 2.4 x 3 centimeters, with moderate surrounding edema with severe mass effect on the fourth ventricle and mild lateral and third dilation with lateral and third ventricles appearing cyst stable to CT head performed yesterday. Edema most severely involves the superior vermis with effacement of the supracerebellar cistern. There is also mild tonsillar ectopia. Given the patient's history, this is consistent with a metastasis. Recommend neurosurgical consultation.

Findings and recommendations discussed with Dr. Spath at 3:40 PM 4/2/2025.